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HP PARACETAMOL TABLET 500 mg

VIPARmi-PC5 (FEO)

DESCRIPTION

Round, orangish-pink film-coated tablet, bevel-edged, shallow convex faces.

COMPOSITION

Paracetamol 500 mg/tablet.

PHARMACODYNAMICS

Paracetamol has analgesic and antipyretic actions but no useful anti-inflammatory properties. It may act by inhibiting prostaglandin synthesis in the central nervous system (CNS) and through a peripheral action by blocking pain-impulse generation. The peripheral action may also be due to inhibition of the synthesis of prostaglandins or to inhibition of the synthesis or actions of other substances, which sensitize pain receptors to mechanical or chemical stimulation.

Paracetamol probably produces antipyresis by acting centrally on the hypothalamic heat-regulating centre to produce peripheral vasodilation resulting in increased blood flow through the skin, sweating, and heat loss. The central action probably involves inhibition of prostaglandin synthesis in the hypothalamus.

PHARMACOKINETICS

Absorption: Rapid and almost complete following oral administration; may be decreased if taken following a high-carbohydrates meal.

Blood Concentration: After an oral dose of 1g to fasting subjects, peak plasma concentration of 20 to 30 µg/ml are attained in about 20 minutes. After giving the same dose to non-fasting subjects, peak plasma concentrations of 10 to 20 µg/ml are attained in 40 to 120 minutes.

After an oral dose of 2g to fasting subjects, peak plasma concentrations of about 30 to 40 µg/ml are attained in 1 to 3 hours.

Half-life: Plasma half-life after therapeutic doses is approximately 1 to 4 hours; does not change with renal failure but may be prolonged in some forms of hepatic disease, in the elderly, and in the neonate, and may be somewhat shortened in children.

Distribution: Widely distributed throughout most body fluids; secreted in the saliva in concentrations paralleling those in the plasma; volume of distribution, about 1 litre/kg body weight.

Protein Binding: Plasma-protein binding is negligible with doses producing plasma concentrations below 60 mcg per ml but increase with increasing concentrations.

Metabolic Reactions: Approximately 90 to 95% of a dose is metabolized in the liver, primarily by conjugation with glucuronic acid, sulfuric acid, and cysteine. The metabolite formed by oxidation, which is hepatotoxic, is conjugated by the glutathione pathway to form mercapturic acid and cysteine conjugates.

Excretion: About 85% of a therapeutic dose is excreted in the urine in 24 hours; of the excreted material in adult, 1 to 4% is unchanged, 20 to 30% is conjugated with sulphate, 40 to 60% is conjugated with glucuronic acid, 5 to 10% is composed of 3-hydroxy-3-sulphate, 3-methoxyglucuronide, and 3-methoxy-3-sulphate metabolites, and 5 to 10% is composed of mercapturic acid and cysteine conjugates. Larger amounts of the mercapturic acid and cysteine conjugates are excreted in overdose.

INDICATIONS

For symptomatic relief of headache, pain and fever.

CONTRAINDICATIONS

Patients hypersensitive to aspirin may not be hypersensitive to paracetamol; however, mild bronchoplastic reactions with paracetamol have been reported in some aspirin-sensitive asthmatics (less than 5% of those tested).

PRECAUTIONS

- Caution in patients with hepatic or renal function impairment.
- Check with physician if symptoms do not improve or if fever lasts longer than 3 days or recurs.
- Chronic use should be avoided, unless otherwise directed by physician.

PREGNANCY AND LACTATION

None known

DRUG INTERACTIONS

Risk of hepatotoxicity with single toxic doses or prolonged use of paracetamol may be increased in chronic alcoholics or in patients regularly taking other hepatic enzyme-inducing agents.

MAIN SIDE/ADVERSE EFFECTS

In recommended therapeutic dosage, paracetamol is usually well tolerated. Rarely, hypersensitivity and haematological reactions may occur.

OVERDOSE AND TREATMENT

Clinical features: Nausea, vomiting, epigastric discomfort, hypoprothrombinaemia, hepatic or renal failure, metabolic acidosis and coma in very severe cases.

Treat overdose by emesis or gastric lavage, if indicated. Administer N-acetylcysteine orally (start within 10 hours after ingestion), 140 mg of 20% solution per kg as a loading dose, followed by 70 mg/kg every 4 hours for 3 days with the necessary supportive treatment if required.

DOSAGE AND ADMINISTRATION

Adults

Oral, 2 tablets every 4 hours, not exceeding 8 tablets daily.

Children 7 to 12 years

Oral, 1 tablet every 4 hours, not exceeding 4 tablets daily.

The information given here is limited. For further information, consult your doctor or pharmacist.

Storage:

Store below 25°C. Protect from moisture.

Presentation/Packing: Film-coated tablet 500 mg x 1000's, Blisters of 10 x 10's, Blisters of 100 x 10's.

Product Registration Holder: HOVID Pharmacy Sdn. Bhd.
121, Jalan Tunku Abdul Rahman (Jalan Kuala Kangsar),
30010 Ipoh, Perak, Malaysia.

Manufactured by: HOVID Bhd.
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31200 Chemor, Perak, Malaysia.

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